Market Access Insight: Jardiva in Italy (2022)

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Access Overview

Reimbursement for Jardiva in Italy was approved on a managed entry agreement requiring real-world evidence submission within 24 months of launch under AIFA.

The regional national health service system applies health technology assessment before listing Jardiva for heart failure with reduced ejection fraction.

Payer Perspectives

Payers in Italy view the price of Jardiva as acceptable only with a demonstrated clinical benefit versus the standard of care.

Formulary placement is reviewed annually, and pricing is linked to outcomes in the registry.

Subgroup analyses suggest that benefit with Jardiva is most pronounced in patients with a favourable biomarker profile and a lower comorbidity burden.

Clinical guidelines in Italy recommend refractory disease use of agents in this class, and recent updates have broadened eligibility based on biomarker status and comorbidity profile.

Access to specialist centres influences adoption: academic centres adopt new therapies faster than community practices, where formulary restrictions and reimbursement criteria apply.

Biosimilar and generic competition in adjacent classes is shaping price expectations, even where Jardiva has no direct substitute.

Combination therapy and sequencing strategies remain debated; respondents noted limited head-to-head evidence between Jardiva and competing agents.

Dose modification guidance for Jardiva is generally followed, but cardiologists said that practical monitoring of hypotension depends on local capacity and patient travel distance.

Cardiologists emphasised that patient-reported outcomes and quality of life are increasingly important when choosing between SGLT2 inhibitor options.

The unmet need in heart failure with reduced ejection fraction persists in patients who relapse or become refractory, where options are limited and outcomes remain poor.

Safety monitoring focused on hypotension and bleeding events, with dose interruption or discontinuation used for severe adverse events.

Patient advocacy groups are pushing for earlier access, and shared decision-making tools are being piloted to explain benefit, risk and treatment burden.

Treatment burden, including monitoring visits and discontinuation after early adverse events, was described as a key factor in long-term adherence and persistence.

Peer-to-peer education and congress data were cited as the most trusted sources when cardiologists evaluate new evidence on SGLT2 inhibitor agents.

Medical education priorities include recognising hypotension early, communicating realistic expectations, and coordinating with primary care.

Evidence Requirements

Required evidence includes overall survival or response rate data, patient-reported outcomes and comparative effectiveness against Jardiva's main competitor.

Regional variation within Italy is notable: large centres report faster uptake and broader use than smaller regional hospitals.

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Guideline committees are expected to review the evidence base for heart failure with reduced ejection fraction again next year, which could change eligibility criteria and referral patterns.

Education needs include biomarker testing, management of adverse events and counselling patients on the expected benefit-risk.

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Pricing and Reimbursement Outlook

A confidential rebate is expected to remain in place until 2024. Market access risk is high for new indications.

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Sequencing after progression remains an open question; cardiologists in Italy reported using a different mechanism of action after Jardiva fails.

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Real-world evidence from registries suggests that outcomes in routine practice are broadly consistent with phase 3 trial results, although persistence and discontinuation rates are higher outside clinical trials.

Registry data show that outcomes in heart failure with reduced ejection fraction depend heavily on early diagnosis, access to specialist care and adherence to maintenance therapy.

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